Regorafenib (Stivarga) monotherapy

Treatment scope · NCCP 00244a This guide covers regorafenib for previously treated metastatic colorectal cancer. Regorafenib is also used in other cancers, but their eligibility and evidence are different. Confirm the treatment setting on your prescription.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Regorafenib oral cycle: Days 1–21, then Days 22–28 off

Active medicines: Regorafenib

Emetogenic risk: Minimal-to-low oral

Medicine	Dose / route	When
Rescue antiemetic	Individual prescription under the oral-systemic anticancer therapy (SACT) pathway	As required, not routine daily prophylaxis NCCP V6 Table 5 offers metoclopramide, prochlorperazine or one 5-HT3 antagonist. Choose and prescribe one appropriate option after contraindication, interaction, renal-function and QT review; do not combine alternatives automatically.

Additional phase-specific support

- No routine premedication is usually required. Provide local hand–foot skin reaction prevention and treatment; examine pressure areas and report pain, blisters or impaired walking early.
- Take the prescribed dose once daily after a light meal containing less than 30% fat, as clarified by current Stivarga SmPC; do not replace a vomited dose or double the next day.
- Check liver tests every two weeks for the first two months, then monthly or as clinically indicated; abnormalities require the separate source-specific closer monitoring plan.
- Check blood pressure weekly for the first six weeks, then before each cycle or as clinically indicated. Baseline thyroid testing; subsequent thyroid, lipase and amylase checks as clinically indicated.
- Review strong CYP3A4/UGT interactions, bleeding, infection, ischaemia, wounds and planned surgery; close international normalised ratio (blood-clotting test) (INR) monitoring if taking warfarin.
- Dose changes require consultant review. HFSR recovery, occurrence count and previous dose determine the pathway; permitted re-escalation is not a patient-led action.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00244 v5 · p2, 6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-05 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required